
HHMI Investigator Inquiry: A RAS-Directed Perioperative Program in Pancreatic Ductal Adenocarcinoma

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Fourteen relevant works between June 2025 and July 2026: two clinical protocols, one Investigational New Drug application, two funding applications, five legislative drafts, and four guidance works. No institutional appointment, no indirect cost, one operator.

August 7, 2026 | Intended partner site: UC San Diego Moores Cancer Center (feasibility stage)

Submitted as an independent scientist under the funding approach set out in Science: A New Golden Age (July 2026). Not medical or regulatory advice, and not endorsed by the FDA, NIH, HHS, HHMI, an IRB, ICH, or any sponsor. UC San Diego is named as the intended partner of choice and is not a sponsor, site, or endorser.

1 Bet on People, Not Just Projects

Science: A New Golden Age asks the roughly **\$200 billion** annual federal research portfolio to “bet on people, not just projects” [1]. Its Chapter II supplies the evidence for why: HHMI-style person-based funding, roughly \$10 million over seven years, produced high-impact publications at nearly double the rate of comparable federally funded peers [2]. The report’s argument is that the difference is not the money but the tolerance for a long horizon and for early failure.

This inquiry asks whether that argument extends to an investigator with no academic appointment. Between June 2025 and July 2026, one person produced fourteen relevant works: the drug was identified from forty pancreatic cancer meta-analyses, three quantitative trial simulations were run and audited, and a complete clinical and regulatory package was written, illustrated in Table 1 [3]. The programme is not a set of aims. It is a single line of inquiry, held for a decade, into whether the RAS pathway can be interrupted around a curative-intent operation in a disease where five-year survival remains in the low teens [4].

What person-based funding buys	What it would buy here
A long horizon, tolerant of early failure	A seven-year run at a question whose first informative readout is four years out
Freedom to change the approach, no new apply	The pathway question is fixed; the agent, the schedule, and the surrogate are not
Selection on the person, not the proposal	Fourteen relevant works, all citable, produced before any award existed

Table 1: Three properties the report attributes to person-based funding, and what each would purchase in this programme. The middle row is the one a project grant cannot supply at all.

2 The Programme, as States Rather Than Aims

A project application lists aims. A person-based application should say what the programme does when an aim fails, which is a question about states and guards (Figure 1).

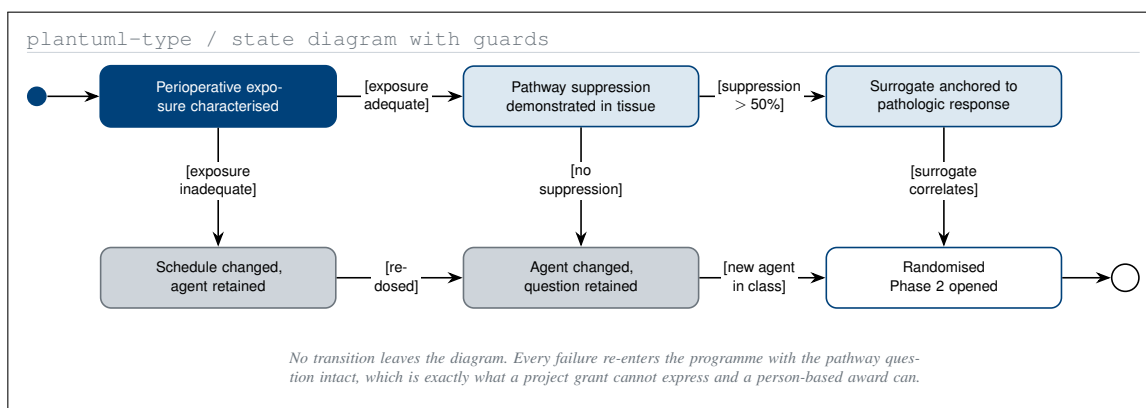


Figure 1: The programme written as states with explicit guards. Every failure transition re-enters the diagram rather than leaving it, because the pathway question survives the loss of any single agent or schedule.

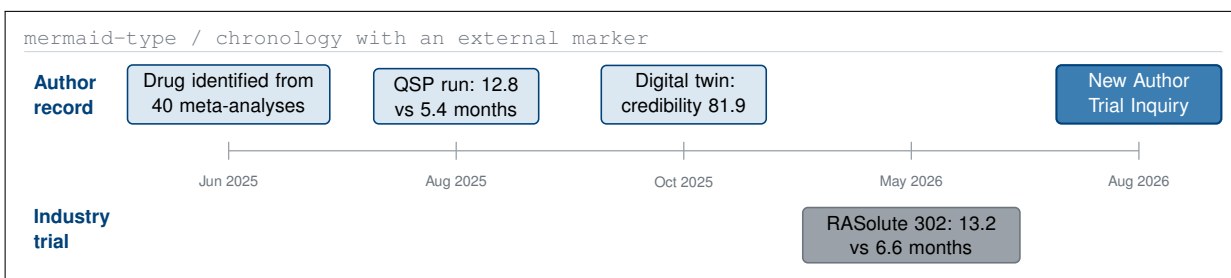


Figure 2: The programme's chronology, with the author's own record above the axis and the one independent readout below it. Separating the bands is what keeps the comparison honest.

3 The Record So Far

Daraxonrasib (RMC-6236) is a RAS(ON) multi-selective inhibitor. RASolute 302 reported median overall survival of **13.2 months** against **6.6** for chemotherapy in the RAS G12 population of previously treated metastatic pancreatic cancer [5]. Nine months earlier, a ten-arm quantitative systems pharmacology simulation with 250 ordinary differential equations per patient had returned **12.8** against **5.4 months**, as depicted in Figure 2. [6]. The proximity is chronology and hypothesis support, not validation: 1000 simulated against 241 enrolled, a combination against a single agent, KRAS G12C against a primarily G12D and G12V population.

The digital-twin work carries an ASME V&V 40 and ICH M15 aligned credibility score of **81.9** across 55 verification notebook tests [7, 8, 9], and each of the three simulations cost an estimated \$36,330 against an industry benchmark of \$120,000 to \$2,000,000 per run.

d2-type / comparison grid

Property	Project grant	Person-based award
Unit of review	Three aims, scored separately	One line of inquiry, held for a decade
Response to a failed aim	New application, 9 to 20 month lead time	Transition in programme, no application
Administrative load	Near half of research time, per the report	One renewal in seven years
Published-impact evidence	Reference rate	Nearly double the reference rate

Figure 3: Person-based against project-based funding on four properties, with the published-impact row carrying the comparison the report itself cites. Only the second row is decisive for this programme.

4 The Clinical Vehicle

The first four years of the programme run through one study: a Phase 1, first-in-human, open-label, single-arm 3+3 dose escalation of perioperative daraxonrasib in up to 18 participants with resectable or borderline-resectable KRAS G12-mutated PDAC, with a staged eight-arm robotic pancreaticoduodenectomy and an on-premises language model confined to advisory output (Table 2) [10, 11]. The operation matters to pharmacology because it is the only setting that yields paired human tumour tissue under controlled timing.

Endpoint	Class	Read at
Dose-limiting toxicity, MTD or RP2D	Co-primary	Each 3+3 cohort, 28-day window
Device- or procedure-related SAEs	Co-primary	30 days after resection
Plasma pharmacokinetics	Secondary	Days -14, -1, and +1
Tumour pathway pharmacodynamics	Secondary	Resection specimen, day 0
ctDNA clearance, pathologic response	Exploratory	Days +30 and +90; specimen at day 0
Overall survival	Exploratory	24 months

Table 2: The endpoint set, ordered by class rather than by interest. The two co-primary endpoints govern the dose decision; every pharmacologic endpoint that motivates the programme is secondary or exploratory.

Positioning note. First in human refers to the integrated surgical and advisory workflow, not to daraxonrasib, which is investigational and already in Phase 3 evaluation. The robotic configuration is specified by manufacturer, model, cleared configuration, arm count, and software version at the site agreement, and no drug supply or letter of authorization is in place with the agent's developer.

5 Seven Years, and the Partner Site

The report's reference point for person-based support (Figure 3) is roughly \$10 million over seven years. This programme's clinical costs are lower than a bench laboratory's: **\$700,000 per year** covers the trial and its verification work [12], and the remainder of a standard investigator allocation would fund the pharmacologic and surrogate work that follows the Phase 1 readout.

Moore's Cancer Center is the intended partner of choice for the clinical work, approached at the feasibility stage only. UC San Diego is not a partner, sponsor, site, or endorser, because no written authorization exists.

Scope of Claims

Claimed: fourteen relevant works produced in fourteen months by one unaffiliated scientist, and a programme whose pathway question survives the loss of any single agent or schedule. Not claimed: that the simulations validate RASolute 302, that the advisory system holds any regulatory clearance, that anything

here has been used in a human, or that UC San Diego has agreed to anything. The trial does not exist until a site, an IRB, an active IND, monitoring, insurance, and a sponsor of record are all in place.

Availability and Conflicts

Every work cited to a DOI is openly deposited; code, verification notebooks, and figure sources are in the repository [13]. The applicant holds no equity in Revolution Medicines or any robotic surgery vendor, and no sponsor, contract research organization, or medical society reviewed this document.

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